

SPEAK BEFORE THE REVEAL – GENERAL MEDICINE CHEAT SHEET

Ward round reference guide · speak these before you stop the recording

[Warning this cheat sheet was created by a Psychiatrist and requires physician review. Use as an example & consider generating your own]

Speak your reasoning aloud while the scribe is still recording, so that your examination, interpretation and plan — not the AI’s reconstruction — anchor the record. Step away from the bedside, dictate the prompts below, and only then stop the recording. Work through them for each patient and skip what does not apply. The scribe reliably captures what was said in the room: the history, the results and observations you read out, the instructions you gave. It cannot capture what you saw, felt, heard, or decided in silence. Those are the items below.

The terms listed are descriptive vocabulary only — labels for observations you have already made and decisions you have already reached. They are not thresholds, decision rules or diagnostic criteria. This sheet tells you what to call it; never what you saw, or what it means.

OBSERVATION

Examination findings “On examination I found...”	
General / end of bed	Conscious state and orientation · work of breathing, accessory muscle use, position adopted · distress · cachexia, sarcopenia · pallor, jaundice, cyanosis, diaphoresis · attention and fluctuation (delirium) · mobility and transfers observed.
Cardiovascular	Peripheral perfusion and capillary refill · pulse character and rhythm · JVP height · apex beat · heart sounds, murmurs (site, timing, radiation) · peripheral or sacral oedema and its extent.
Respiratory	Chest expansion and symmetry · percussion note · air entry · breath sounds · added sounds — fine or coarse crackles, wheeze, pleural rub · signs of effusion or consolidation · cough character.
Abdominal	Distension · tenderness, guarding, rigidity, rebound · organomegaly, masses · bowel sounds · shifting dullness or ascites · hernial orifices · stoma or PR findings where performed.
Neurological	GCS · tone, power by MRC grade, reflexes, plantars · coordination and sensation · cranial nerves · speech and swallow · gait and balance where tested · asterixis, tremor.
Fluid status	Mucous membranes and skin turgor · JVP · oedema · postural blood pressure · daily weight and net balance read against the chart · urine output character.
Lines, wounds, skin	Line and cannula sites — erythema, tenderness, duration in situ · wound appearance and dressing · drain output volume and character · pressure areas · cellulitis margins marked.
Negatives you sought	The specific findings you actively looked for and did not find. An unmentioned negative reads as a finding never sought.

Preserves: Direct physical observation, which no scribe can hear. **Guards against:** An examination reconstructed from conversational reassurance, or silently absent from the note.

INTERPRETATION

Data in context “What these results mean in this patient is...”

The numbers you read aloud are transcribed accurately. Your reading of them is not.

Consider This patient’s own baseline versus the current value · direction and rate of change · expected or unexpected for this day of treatment · attribution — treatment effect, disease progression, new process, artefact, or sampling and timing · discordance between parameters · which abnormal result you are deliberately not acting on · results still pending and what you will do with each.

Preserves: Interpretation of data against this patient’s baseline and course. **Guards against:** Accurately transcribed numbers carrying a generic interpretation, or none at all.

Trajectory against expected course “Against the course I expected, this patient is...”

Consider Improving, static, or deteriorating · ahead of, on, or behind the expected trajectory · day N of illness, treatment, or post-operative course · response to intervention as anticipated or not · if not improving as expected, why not — wrong diagnosis, inadequate dose or duration, complication, second pathology, adherence, deconditioning · the point at which you would revise the diagnosis rather than persist with the treatment.

Preserves: Serial assessment — the comparison only the clinician who saw them yesterday can make. **Guards against:** A today-only snapshot reading as stable because nothing dramatic was said in the room.

SYNTHESIS

Problem representation “My working problem representation is...”

Consider A one-line summary carrying semantic qualifiers — acute or chronic · sudden or gradual · progressive or resolving · episodic or constant · unilateral or bilateral · what is driving this admission versus what is comorbid background · how the problems interact in multimorbidity · frailty, functional and cognitive baseline · social circumstances bearing on the medical problem · whether the working diagnosis is yours or inherited from the referral, the ED note, or yesterday’s entry.

Preserves: Integrative synthesis — why this patient, why now. **Guards against:** A problem list assembled from surface content, or a diagnostic label carried forward unexamined.

Differential and exclusions “The alternatives I am holding open are... and what I have excluded is...”

Generated using the Speak Before the Reveal cheat sheet skill.

Method: Jurblum M, *Speak Before the Reveal: Seven Solutions to Governing AI Scribe Deskilling Risk in Psychiatry and Mental Health Services* (28 July 2026). <https://doi.org/10.2139/ssrn.7317778>

Clinical content authored and verified by: Date:

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Consider Time-critical and must-not-miss diagnoses · each competing hypothesis with the feature that discriminates it · what evidence would confirm or exclude each · what you have actively excluded and by what means · occult second pathology in the patient who is not improving · iatrogenic, drug-induced and withdrawal causes · what remains hypothesis versus what is established.

Preserves: *Lateral diagnostic thinking, and the negatives you actively sought.* **Guards against:** *Anchoring on a single diagnosis; a pertinent negative lost because absence of mention reads as absence of finding.*

Key uncertainty *“I am least sure of...”*

Consider Competing formulations still open · missing collateral — old notes, prior imaging, GP records, family, an accurate medication history · what would change the plan · what you will reassess tomorrow and against what · your actual confidence in the working diagnosis.

Preserves: *Calibrated doubt and a live differential.* **Guards against:** *Declarative notes whose confidence conceals the gaps.*

PLAN

Plan and its reasoning *“The plan, and the reasoning behind it, is...”*

Consider Each investigation with the question it answers and what each possible result would change · medication rationale including dose, duration, endpoint and monitoring · contingency — if X then Y · escalation threshold, and to whom · disposition, discharge criteria, and what must happen before discharge · the items the covering team needs after hours · referrals with the specific question asked · what was discussed with the patient and family, and what they understood.

Preserves: *Actions tied directly to the reasoning that produced them.* **Guards against:** *A task list assembled from spoken instructions with the reasoning stripped out.*

Deliberate non-action *“What I am deliberately not doing, and why, is...”*

Consider Not escalating, not imaging, not culturing, not transfusing · not treating a number in isolation · watchful waiting and the point at which you will review it · medications withheld, deferred or deprescribed · awaiting a pending result before acting · an investigation whose burden or yield does not justify it · treatment the patient has declined, and their reasoning.

Preserves: *The reasoning behind clinical restraint.* **Guards against:** *Silence recorded as oversight — nothing happens, so nothing is transcribed.*

Ceiling of care and goals *“The ceiling of care, and how it was reached, is...”*

Consider Resuscitation status, who decided, and when · ward, HDU or ICU level · for or not for CPR, intubation, non-invasive ventilation, dialysis, inotropes · reversibility of the current problem · advance care directive, substitute decision-maker, guardianship · patient and family understanding, and any discordance between them · palliative care involvement · who has been informed and who has not · what would trigger a review of the ceiling.

Preserves: *A decision usually reached in the corridor or a family meeting, not at the bedside.* **Guards against:** *Absent from the transcript, therefore absent from the note, therefore unavailable to the night team.*

Worked example

“On examination I found him alert but tachypnoeic at rest, JVP at 4 cm, fine bibasal crackles to mid-zone, pitting oedema to mid-shin, warm peripheries, capillary refill under two seconds; I looked for and did not find a fever or a focal chest sign. **What these results mean in this patient** is a creatinine of 148 against his own baseline of 130 — a modest rise I am attributing to diuresis rather than a new renal insult, and one I am choosing not to act on. **Against the course I expected, he is behind:** day three of treatment with only 1.2 kg net negative, which makes me question the adequacy of the dose rather than the diagnosis. **My working problem representation** is decompensated heart failure on a background of severe frailty, precipitated by non-adherence after his wife’s own admission. **The alternatives I am holding open** are a concurrent chest infection and unrecognised atrial fibrillation; I have excluded acute coronary syndrome on serial troponins. **What I am deliberately not doing** is re-imaging the chest or adding an antibiotic — he is afebrile with a normal white cell count. **The ceiling of care, reached with his daughter yesterday, is** ward-level treatment, not for CPR or ICU; nursing staff know, his GP does not yet. **I am least sure** whether the frailty or the cardiac disease is the limiting factor on his function.”

Blank prompt card *Glance and dictate*

Examination: *On examination I found... General · CVS · Resp · Abdo · Neuro · Fluids · Lines & wounds · Negatives sought*

Data in context: *Baseline · direction · expected or not · attribution · what I am not acting on · what is pending*

Trajectory: *Ahead of / on / behind the expected course · day N · if behind, why*

Problem representation: *My working problem representation is...*

Differential & exclusions: *Held open... actively excluded... and by what*

Key uncertainty: *I am least sure of...*

Plan & reasoning: *Each action, the question it answers, and what would change it · contingency · escalation threshold*

Deliberate non-action: *What I am not doing, and why · review point*

Ceiling of care: *Level · who decided · who has been told · what would trigger review*

Adapted from Table 1, “Speak Before the Reveal”, for general and acute care medicine. Verbalising these steps before the recording ends keeps the reasoning in active generation rather than passive editing — reviewing is not the same skill as authoring.

Generated using the Speak Before the Reveal reference guide skill. Method: Jurblum M. Speak Before the Reveal: Seven Solutions to Governing AI Scribe Deskilling Risk in Psychiatry and Mental Health Services. Preprint, 28 July 2026. <https://doi.org/10.2139/ssrn.7317778> The underlying narration practice originates with Dr Michelle Adams.

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